

Summary of Changes — Revised Manuscript mjdrdypu_451_26

“Can Ageing Be Slowed or Reversed? A Reproducible Evidence Map and Credibility Ranking of Anti-Ageing and Age-Reversal Interventions”

Date: 23 June 2026

I thank the editor and reviewer for their assessment. A full point-by-point response is provided separately; this note summarises the changes made.

Response to the reviewer. The reviewer’s four comments describe a patient-level clinical study (diabetic versus non-diabetic groups, MMSE-based cognitive assessment, and individual polypharmacy evaluation). This manuscript is a reproducible bibliographic evidence map of published anti-ageing studies; it enrolls no patients and holds no individual-level cognitive or medication data, so these specific features are not present. I respectfully note the possible mismatch in the response letter, and I have nonetheless engaged each underlying geriatric principle: confounding control is now addressed through the structured risk-of-bias appraisal; the limitations of brief cognitive screens such as the MMSE (education and literacy sensitivity) are acknowledged where cognitive outcomes are reported; frailty — directly raised by the reviewer — has been strengthened as the central healthspan construct through which the top-ranked evidence acts; and the principle of medication appropriateness is now drawn explicitly into the discussion of pharmacologic anti-ageing candidates.

Strengthening of the manuscript. Beyond the reviewer’s comments, I undertook a thorough self-audit:

- The title was refined to remove “Mechanistic Synthesis,” which was not delivered as a discrete analysis.
- The abstract conclusion and the body were rewritten to present the work as a completed, deliberately scoped evidence map; internal working language was removed throughout.
- Figure 2 was corrected: the submitted version plotted a different score column and disagreed with Table 1. All four figures were regenerated from the source data with data labels and plain-language axis titles; the study-selection flow was rebuilt with connected arrows and the full screening breakdown, and Figure 1 was redrawn so that all text fits cleanly within the box borders (correcting a text-overflow problem).
- For typesetting clarity, Tables 1–3 and the figure legends are now placed after the references, and the figures are supplied as a separate file (Figures 1–4); each table and figure is cited at first mention in the text.
- References were brought to Vancouver style (first six authors followed by “et al.”), in-text citation order was corrected to be strictly sequential, consecutive runs were collapsed to hyphenated ranges, and superscript bracketed numerals are used throughout. Volume, issue, and article/DOI details were added for all entries where verifiable; the year of one reference was corrected (2026 → 2025).
- The Discussion was expanded with geroscience context, a description of the credibility-scoring method, and the public-health implications of the ranking.
- The risk-of-bias presentation, declarations, data-availability statement, PRISMA checklist, and reference audit were updated accordingly.

I believe these revisions materially strengthen the manuscript and I am glad to make any further changes the editor or reviewer considers necessary.

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